



EVALUATION 6 — CATASTROPHICALLY COMPLEX

Primary Diagnosis: Quadriplegia (Incomplete C4-C5 SCI) with Ventilator Dependence, Tracheostomy, PEG Tube, Neurogenic Bladder/Bowel, Autonomic Dysreflexia, and Psychiatric Crisis History

PATIENT DEMOGRAPHICS

Field Detail

Patient Name Darius Malik Thompson

Date of Birth 02/14/1995

Age 30

Sex Male

Race/Ethnicity Black / African American

Primary Language English

Interpreter Needed No

Marital Status Single

Medical Record Number CI-2025-00455

Address of Service 1122 Magnolia Court, Unit 1 (ground-floor ADA accessible), San Carlos, CA 94070

Cell Phone (650) 555-0233 (voice-activated; patient can use with adaptive mount)

Email <darius.thompson95@email.com>

County San Mateo

EMERGENCY CONTACTS

Priority Name Relationship Phone Lives with Patient?

1 Angela Thompson Mother (650) 555-0234 (home) / (650) 555-0157 (cell) Yes — primary caregiver (took leave from work; FMLA exhausted; now unpaid leave)

2 Terrence Thompson Father (650) 555-0235 No — divorced from mother; sees patient weekly; provides financial support

3 DeShawn Carter Best friend / informal caregiver (trained at rehab) (650) 555-0892 No — but stays overnight 2-3 nights/week to give mother respite

LIVING SITUATION

Field Detail

Lives With Mother Angela (age 58, healthy, dedicated but experiencing severe caregiver burnout — lost 20 lbs in 3 months, insomnia, crying episodes per self-report)

Residence Type Ground-floor ADA-accessible apartment; recently modified (roll-in shower, widened doorways, Hoyer ceiling track lift installed, hospital bed with specialty mattress)

Access Level entry (no steps); automatic door opener installed

Pets None

DME in Home (extensive — see separate section below)

DME INVENTORY

Equipment Details

Ventilator (primary) Trilogy Evo (Philips); settings: AC/VC, TV 550, RR 14, PEEP 5, FiO2 30%; patient on vent 24/7 with brief (15-30 min) trials off vent during day per pulmonology weaning protocol
Ventilator (backup) LTV 1200; same settings; kept charged at bedside at ALL times
Suction Machine (primary) DeVilbiss portable; bedside
Suction Machine (backup) Manual (DeLee) suction — for power outage/emergencies
Pulse Oximeter Continuous monitoring at night; spot-check during day
Tracheostomy Supplies Shiley #8 cuffed trach; extra trach at bedside (same size + one size smaller — #6 — for emergency); trach care kit; suction catheters (14 Fr); NS bullets; trach ties; inner cannula replacements
Ambu Bag (BVM) At bedside at ALL times — for emergency manual ventilation
Hospital Bed Full-electric with Clinitron-type alternating pressure/low air loss mattress (Braden 9 — very high risk)
Hoyer Ceiling Track Lift Installed in bedroom → bathroom path; slings x3
Tilt-in-Space Wheelchair Permobil M5 Corpus; joystick abandoned (cannot operate) — currently using sip-and-puff control; headrest, lateral supports, hip guides, custom cushion (ROHO Quadro Select)
Shower/Commode Chair Roll-in shower chair with tilt function
PEG Tube Supplies Mic-Key 20 Fr low-profile button; extension sets; 60 mL syringes; Kangaroo ePump for continuous feeds
Catheter Supplies Foley catheter (16 Fr, 10 cc balloon); leg bags; night drainage bags; catheter care supplies; backup Foley kits x3
Bowel Program Supplies Digital stimulation gloves, suppositories (bisacodyl), lubricant, chux pads, gloves, bowel program checklist
Communication Eye-gaze communication device (Tobii Dynavox I-Series); voice-activated smartphone mount; Amazon Echo (voice-activated home controls); call bell (pressure-sensitive pad by left cheek — one of few volitional movements)
Emergency Portable generator (for vent power during outage); UPS battery backup (connected to primary vent — provides 2 hrs); 911-registered address (flagged for vent-dependent patient)

PAYER INFORMATION

Priority Payer ID / Policy Number
Primary Medicare Part A (SSDI) 4RB3-TW7-NX28
Secondary Medicare Part B Same
Tertiary Medi-Cal (QMB — pays Medicare cost-sharing) 94062209D

PHYSICIAN INFORMATION

Role Name Specialty Phone
Attending / Certifying Dr. Raymond Choi Physical Medicine & Rehabilitation (PM&R) (650) 555-1020
Pulmonologist Dr. Sarah Atkins Pulmonary / Critical Care (650) 555-1135
Urologist Dr. David Moran Urology (Neurourology) (650) 555-1245
Psychiatrist Dr. Nina Kovacs Psychiatry (650) 555-1780
PCP Dr. Evelyn Nakamura Family Medicine (650) 555-0400
Wound Care Dr. Angela Torres Wound Care (650) 555-1550
GI Dr. Heather Kim Gastroenterology (650) 555-1320
Pain Dr. Sanjay Patel Pain Medicine (650) 555-1445
F2F Encounter Date: 06/10/2025 F2F Performed By: Dr. Raymond Choi

REFERRAL / HOSPITALIZATION HISTORY

Event Facility Dates Reason

Acute Hospitalization Stanford Medical Center 03/02/2025 – 03/28/2025 Motor vehicle accident → C4-C5 burst fracture with incomplete SCI (ASIA B); anterior cervical corpectomy and fusion C4-C5 with posterior instrumentation C3-C6; tracheostomy (ventilator-dependent); PEG tube placement; Foley catheter placement

Inpatient Rehabilitation Santa Clara Valley Medical Center SCI Unit 03/28/2025 – 06/12/2025 Comprehensive SCI rehabilitation (76 days); complicated by: (1) autonomic dysreflexia episodes x4 (urinary catheter obstruction x2, fecal impaction x1, ingrown toenail x1); (2) MRSA pneumonia (VAP) — treated; (3) Stage 3 pressure injury sacrum (developed in acute care, improved to Stage 2 in rehab); (4) psychiatric crisis — acute suicidal ideation 05/15/2025 ("I want to die, I can't live like this") requiring 1:1 observation x 72 hrs and psychiatric consultation
Start of Care Date: 06/13/2025

DIAGNOSES (30+ active diagnoses)

Priority ICD-10 Diagnosis

Primary G82.51 Quadriplegia, C4-C5 level, incomplete (ASIA B — sensory incomplete, motor complete below injury)

Acute S12.491A Other fracture of fifth cervical vertebra, initial encounter (status post surgical fusion)

Active Z99.11 Dependence on respirator (ventilator) — 24 hours

Active Z93.0 Tracheostomy status (Shiley #8 cuffed)

Active J95.00 Tracheostomy complication, unspecified (history of granulation tissue at stoma — treated in rehab; monitoring)

Active Z93.1 Gastrostomy status (PEG — Mic-Key 20 Fr)

Active Z96.89 Presence of other specified functional implants (cervical spinal fusion hardware)

Active G95.20 Unspecified cord bladder (neurogenic bladder — areflexic; Foley catheter dependent)

Active K59.2 Neurogenic bowel, not elsewhere classified (areflexic bowel; requires bowel program)

Active G90.4 Autonomic dysreflexia (episodic — MEDICAL EMERGENCY when triggered)

Active L89.153 Pressure ulcer of sacral region, stage 3 (improved to Stage 2 in rehab; currently 3.0 cm x 2.5 cm x 0.4 cm depth, 80% granulation)

Active L89.152 Pressure ulcer of sacral region, stage 2 (coding note: use current stage)

Active Z87.39 History of MRSA pneumonia (ventilator-associated, 04/2025 — treated, resolved)

Active J96.11 Chronic respiratory failure with hypoxia (SCI-related restrictive lung disease; baseline SpO2 93-95% on vent)

Active J98.11 Atelectasis (recurrent bilateral lower lobe; requires aggressive pulmonary toilet)

Active G89.4 Chronic pain syndrome (neuropathic pain below injury level — burning, tingling, shooting)

Active M62.3 Immobility syndrome (contracture prevention needed — bilateral UE and LE)

Active E44.0 Moderate protein-calorie malnutrition (albumin 2.6, weight loss)

Active D62 Acute posthemorrhagic anemia (Hgb 9.4 — post-surgical, chronic disease)

Active N39.0 Urinary tract infection (ESBL E. coli — treated, completing oral antibiotics)

Active I82.4Z9 Acute embolism and thrombosis of unspecified deep vein of lower extremity (DVT right popliteal — 04/2025, on warfarin)

Active Z79.01 Long-term anticoagulant use (warfarin — target INR 2.0-3.0)

Active F32.2 Major depressive disorder, single episode, severe without psychotic features

Active F43.10 Post-traumatic stress disorder (MVA-related; flashbacks, nightmares, hyperarousal)

Active R45.851 Suicidal ideation (history — 05/15/2025; currently denying but remains HIGH RISK; safety plan in place)

Active E87.6 Hypokalemia (PEG feed-related; supplemented)

Active E55.9 Vitamin D deficiency (immobility, no sun exposure)

Active M80.08XA Age-related osteoporosis with pathological fracture (high risk — SCI-related bone loss; DEXA pending)

Active G24.9 Dystonia, unspecified (spasticity, bilateral lower extremities — on baclofen pump evaluation list)

Active R47.1 Dysarthria (mild — can speak with Passy-Muir valve in place when cuff deflated; limited endurance)

Active R63.4 Abnormal weight loss

ALLERGIES

Allergen Reaction Severity

Latex Anaphylaxis LIFE-THREATENING — ALL supplies must be latex-free

Fluoroquinolones (all) Tendon rupture (Achilles, bilateral — 2022; pre-existing on ciprofloxacin) Severe

Tramadol Seizure Severe

Adhesive tape Skin tears / MARSII Moderate

CURRENT MEDICATIONS (32 medications)

Medication Dose Route Frequency Purpose Critical Notes

- 1 Enteral formula (Jevity 1.5) 70 mL/hr PEG (continuous via pump) 18 hrs/day (6 AM – 12 AM; off 12 AM – 6 AM) Primary nutrition 1890 kcal/day; supplemented with protein powder; free water flushes 200 mL Q4H
- 2 Baclofen 20 mg PO (via PEG) QID Spasticity Intrathecal pump evaluation pending
- 3 Gabapentin 600 mg PO (via PEG) TID Neuropathic pain
- 4 Oxycodone IR 5 mg PO (via PEG) Q6H PRN severe pain (max 20 mg/day) Breakthrough pain Controlled substance. Mother administers and tracks.
- 5 Warfarin 7.5 mg PO (via PEG) Daily (5 PM) DVT treatment INR monitoring 2x/week. Multiple drug interactions with PEG meds.
- 6 Sertraline 150 mg PO (via PEG) Daily AM Depression / PTSD
- 7 Mirtazapine (Remeron) 30 mg PO (via PEG) HS Depression (adjunct) + appetite stimulation + insomnia
- 8 Prazosin 2 mg PO (via PEG) HS PTSD-related nightmares Monitor BP (alpha-blocker — orthostatic risk less relevant in wheelchair, but can affect baseline BP)
- 9 Lorazepam 0.5 mg PO (via PEG) PRN acute anxiety/panic (max BID) Anxiety / PTSD flashbacks Controlled substance.
- 10 Oxybutynin 5 mg PO (via PEG) TID Neurogenic bladder — reduce detrusor spasticity
- 11 Methenamine hippurate (Hiprex) 1 g PO (via PEG) BID UTI prophylaxis (recurrent)
- 12 Cranberry extract 500 mg PO (via PEG) BID UTI prophylaxis (adjunct)
- 13 Nitrofurantoin (Macrobid) 100 mg PO (via PEG) BID x 7 days (through 06/20/2025) Current ESBL E. coli UTI treatment (sensitivity-confirmed)
- 14 Bisacodyl suppository 10 mg PR Every other day (MWF) — bowel program Neurogenic bowel management Administer 15-30 min before digital stimulation
- 15 Docusate/Senna 100/8.6 mg PO (via PEG) BID Bowel program — stool softener/stimulant
- 16 Polyethylene glycol (MiraLAX) 17 g PO (via PEG) Daily Bowel program — osmotic
- 17 Albuterol 2.5 mg/3 mL INH (nebulizer via vent circuit) Q4H and PRN Bronchodilation; secretion mobilization
- 18 Ipratropium 0.5 mg/2.5 mL INH (nebulizer via vent circuit) QID Bronchodilation; reduce secretions
- 19 Acetylcysteine (Mucomyst) 10% 3 mL INH (nebulizer via vent circuit) BID Mucolytic — thin secretions May trigger bronchospasm — give albuterol first
- 20 Normal Saline nebulization 3 mL INH (nebulizer via vent circuit) Q4H between other nebs Secretion hydration
- 21 Omeprazole 20 mg PO (via PEG) Daily GERD / GI prophylaxis
- 22 Warfarin — already listed above
- 23 Calcium + Vitamin D 600/400 PO (via PEG) BID Bone health
- 24 Vitamin D3 2000 IU PO (via PEG) Daily (in addition to calcium/D) Deficiency
- 25 Multivitamin (liquid) 15 mL PO (via PEG) Daily General
- 26 Zinc 50 mg PO (via PEG) Daily Wound healing
- 27 Vitamin C 500 mg PO (via PEG) BID Wound healing
- 28 Potassium chloride (liquid) 20 mEq PO (via PEG) BID Hypokalemia
- 29 Ferrous sulfate (liquid) 300 mg PO (via PEG) Daily Anemia Give 2 hrs apart from omeprazole and calcium
- 30 Melatonin 5 mg PO (via PEG) HS Sleep cycle regulation
- 31 Acetaminophen 650 mg PO (via PEG) Q6H PRN Pain / fever
- 32 PRN autonomic dysreflexia kit Nifedipine 10 mg bite-and-swallow (via PEG) or 2% nitroglycerin paste 1 inch topical PO or Topical
EMERGENCY USE ONLY for AD episode (SBP >150 or symptoms) LIFE-SAVING. See AD Emergency Protocol. Must be immediately accessible at all times — bedside, in wheelchair bag, and in bathroom.

AUTONOMIC DYSREFLEXIA — EMERGENCY PROTOCOL

THIS IS A MEDICAL EMERGENCY. Autonomic dysreflexia (AD) in this patient can be FATAL (hypertensive crisis → stroke, seizure, cardiac arrest).

Signs/Symptoms

Sudden severe headache ("pounding")
BP >150/90 (patient's baseline is ~105/65 — ANY SBP $\geq$ 140 is concerning)
Flushing/sweating above the level of injury (face, neck, chest)
Piloerection ("goosebumps") above injury
Nasal congestion
Blurred vision
Bradycardia (may be severe — HR <50)
Anxiety, feeling of "impending doom"
IMMEDIATE ACTIONS (DO NOT WAIT FOR ORDERS):

Sit patient upright immediately (lower legs if possible) to use orthostatic effect to reduce BP.
Loosen all restrictive clothing, abdominal binder, compression stockings.
Check BP immediately — if SBP $\geq$ 150, proceed to pharmacologic intervention.

Identify and remove the trigger

Bladder (most common): Check Foley catheter — is it kinked, clamped, obstructed? Irrigate gently with 30 mL NS. If no flow, replace Foley immediately. If straight cath needed, use lidocaine gel.
Bowel: Check for fecal impaction (apply lidocaine jelly to rectal area first to prevent worsening AD, then gently check). If impacted, disimpact carefully.
Skin: Check for pressure injury, ingrown toenail, tight clothing, strap/belt compression, temperature extremes, tight catheter leg strap.
Other: UTI, DVT, fracture (can occur with osteoporotic SCI bone from minor trauma), abdominal pathology.
If trigger cannot be identified/resolved AND SBP remains $\geq$ 150:
Administer nifedipine 10 mg — bite capsule and administer contents via PEG (rapid onset).
OR apply 2% nitroglycerin paste 1 inch to forehead or chest ABOVE the level of injury. Remove when BP normalizes.
Monitor BP every 2-5 minutes until resolved.
Call 911 if:
SBP >180 and not responding to interventions within 5 minutes.
LOC changes.
Seizure activity.
Severe bradycardia (HR <40).
Symptoms worsen despite interventions.
Notify physician — Dr. Choi or on-call provider.
Document — full event documentation including time of onset, symptoms, BP readings, interventions, trigger identified, resolution time.
AD history in this patient: 4 episodes in rehab. Most recent: 05/30/2025 (fecal impaction; SBP peaked at 195/110; resolved with disimpaction + nifedipine). This patient is at HIGH RISK for recurrent AD.

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter Value Notes
BP 108/62 Baseline for this patient (SCI hypotension — normal)
HR 64 (baseline bradycardia — SCI-related)
RR 14 (ventilator-set) + 0-2 spontaneous over-breaths
Temp 98.8°F WNL
SpO2 94% on vent (FiO2 30%) Baseline for this patient
Weight 152 lbs (69 kg) — pre-injury weight 195 lbs Loss of 43 lbs (22%) in 3 months
Pain 7/10 neuropathic (bilateral LE — "burning, like fire ants"); 4/10 neck/surgical site
RASS 0 (alert and calm)

PRESSURE ULCER ASSESSMENT

Sacral Pressure Ulcer

Parameter Finding

Location Sacrum/coccyx

Stage Stage 2 (was Stage 3 in acute care; epithelialized to Stage 2 in rehab)

Dimensions 3.0 cm L x 2.5 cm W x 0.4 cm D

Wound Bed 80% granulation (beefy red), 20% epithelializing edges

Drainage Scant serous

Peri wound Intact, mild maceration at inferior edge

Wound Care Orders Cleanse NS; apply Mepilex Border Sacrum foam dressing; change every 3 days and PRN soiling; off-load pressure — repositioning Q2H minimum; specialty mattress continuous

Contributing Factors Immobility, malnutrition (albumin 2.6), incontinence exposure (bowel program imperfect — occasional incontinence between programs), moisture

Braden Scale 9 (VERY HIGH RISK) — Sensory: 1 (completely limited); Moisture: 3 (occasionally moist); Activity: 1 (bedfast); Mobility: 1 (completely immobile); Nutrition: 2 (probably inadequate); Friction/Shear: 1 (problem)

Additional Skin Surveillance Areas

Bilateral ischial tuberosities (wheelchair sitting) — currently intact; ROHO cushion in place; weight shifts via tilt every 15-30 min

Bilateral heels — currently intact; heel suspension boots in bed; float heels off mattress

Bilateral greater trochanters — currently intact; repositioning schedule

Occiput — currently intact; gel pillow

Ears — trach ties/vent tubing pressure; currently intact; pad tubing

Right lateral malleolus — mild redness, blanching — pre-Stage 1; reposition and monitor

HOMEBOUND STATUS

CONFIRMED. Ventilator-dependent quadriplegic requiring Hoyer lift for all transfers, continuous mechanical ventilation, suctioning, and attendant care for all ADLs. Unable to leave home without specialized wheelchair-accessible transport, portable ventilator, suction equipment, and at minimum one trained attendant. Taxing effort is extreme. Absences are exclusively for medical appointments (infrequent — most specialists will do telehealth visits for this patient).

ADVANCE DIRECTIVES

Document Status

AHCD Yes — executed 06/2025 at rehab. Mother Angela designated as primary agent. Father Terrence as alternate. Specific instructions: Patient wants full treatment including ventilator (already on vent); does NOT want CPR continued beyond 45 minutes if no response; wants comfort measures ONLY if physician determines he is in permanent vegetative state.

POLST Yes — Section A: Full Treatment; Section B: Full treatment including intubation (already trached); Section D: Long-term artificial nutrition — yes (already on PEG feeds).

Mental Health Advance Directive Yes — executed 06/2025. Designates mother Angela as mental health agent. Specific instructions: If experiencing acute psychiatric crisis with suicidal ideation, patient authorizes: (1) psychiatric hospitalization if recommended by Dr. Kovacs; (2) medication adjustment including antipsychotics if needed; (3) 1:1 observation; (4) does NOT authorize ECT under any circumstances.

Safety Plan (Suicide Prevention) On file — developed 05/18/2025 at rehab with Dr. Kovacs. Copy in clinical record. Includes: warning signs, coping strategies, reasons for living (mother, friend DeShawn, desire to mentor other SCI patients), emergency contacts (988 Suicide & Crisis Lifeline; Dr. Kovacs' answering service; 911).

PSYCHIATRIC / SUICIDE RISK ASSESSMENT

This patient is at ELEVATED AND ONGOING RISK for suicidal ideation and self-harm.

Risk Factor Present? Details

Prior suicidal ideation YES 05/15/2025 — active SI with plan ("I could tell my mom to turn off the vent") requiring 1:1 observation x 72 hrs
Access to lethal means YES Ventilator dependence itself is the means — patient could potentially direct caregiver to disconnect, or refuse ventilator if competent. Medications also accessible.
Recent major loss/life change YES Catastrophic SCI with quadriplegia at age 30; loss of independence, career (was a personal trainer), romantic relationship (girlfriend left), financial security, housing
Chronic pain YES Severe neuropathic pain — significant suffering
Depression diagnosis YES MDD, severe
PTSD YES MVA-related flashbacks, nightmares
Social isolation PARTIAL Mother is devoted; friend DeShawn visits; father visits weekly. BUT patient reports feeling "trapped" and "like a burden." Lost most of his social circle post-injury.
Male, young adult YES Demographic risk factor
Substance use NO No history
Current SI NO (assessed 06/13/2025) "Not right now. I'm not in that place anymore. But I'm not going to lie and say it won't come back."
PHQ-9 22 (severe)
PCL-5 (PTSD) 58 (probable PTSD; >33 = clinical threshold)
Columbia Suicide Severity Rating Scale (C-SSRS) Ideation Type: 3 (active SI with method, no intent, no plan — at time of most recent assessment); Intensity: moderate Baseline established for ongoing monitoring

Psychiatric Monitoring Plan

PHQ-9 every 2 weeks (RN or MSW administered)
C-SSRS screening every visit (RN)
If SI reported: activate safety plan; contact Dr. Kovacs immediately; do NOT leave patient unattended; assess lethality and intent; call 988 or 911 if imminent risk
MSW weekly visits for therapeutic support, coping skills, adjustment counseling
Coordinate with Dr. Kovacs for medication management (monthly psychiatry telehealth visits scheduled)
Mother and DeShawn trained on warning signs and safety plan activation

SERVICES ORDERED

Service Discipline Frequency Duration
Skilled Nursing RN Daily x 4 weeks (vent management, trach care, wound care, PEG management, catheter care, bowel program supervision, labs, teaching, psychiatric monitoring), then 3-5x/week ongoing Indefinite (complex, vent-dependent; anticipate ongoing need for 6+ months minimum)
Physical Therapy PT 5x/week x 4 weeks, then 3x/week ongoing Indefinite
Occupational Therapy OT 5x/week x 4 weeks, then 3x/week ongoing Indefinite
Speech-Language Pathology SLP 3x/week x 4 weeks, then 2x/week 8 weeks (reassess)
Medical Social Work MSW 2x/week x 4 weeks, then 1x/week ongoing Indefinite
Home Health Aide HHA BID (AM + PM personal care, repositioning, equipment assist) Indefinite
Respiratory Therapy (if available through agency; otherwise coordinated with DME vendor) RT 2x/week x 4 weeks, then 1x/week As needed
SKILLED NURSING FOCUS AREAS (highest complexity level)
Ventilator Management — Verify settings each visit. Assess respiratory status (breath sounds, SpO2, ETCO2 if available, WOB, secretion characteristics). Vent circuit change per schedule. Troubleshoot alarms. Coordinate with pulmonology for weaning trials. Emergency protocol: vent failure → disconnect from vent → manual bag ventilation with Ambu bag → assess for obstruction → call 911 if cannot reestablish ventilation. Backup vent ready at all times.

Tracheostomy Care — Trach site assessment each visit (stoma integrity, granulation tissue, infection signs). Trach care (clean stoma, change inner cannula, change trach ties) per protocol. Suctioning PRN (hyperoxygenate before/after; note: SCI patient may not cough effectively — suction may be frequent). Monitor for trach complications: tube displacement, obstruction, tracheoesophageal fistula, tracheoinnominate fistula (life-threatening — massive hemorrhage; apply direct pressure, call 911 immediately), granulation tissue, stomal infection. Passy-Muir valve application for speech during cuff-deflation trials per SLP/pulmonology. Emergency trach change protocol (spare trach at bedside).

Pulmonary Toilet — Nebulizer treatments per schedule. Chest physiotherapy (manual or mechanical insufflation-exsufflation — "CoughAssist" if ordered). Secretion management. Atelectasis prevention (position changes, deep breathing via vent sigh breaths, nebulizers). Monitor for pneumonia (changed sputum color/volume, fever, declining SpO2, infiltrate on CXR).

PEG Tube Management — Site assessment each visit (skin integrity, granulation, leakage, buried bumper signs). Flush per protocol (30 mL water before/after meds, Q4H during feeds, before/after feed interruptions). Medication administration via PEG — verify crushability of each medication; stagger medications that interact; flush between medications. Monitor tolerance (residuals if ordered, abdominal distention, nausea, diarrhea). Coordinate with RD on formula adjustments.

Pressure Ulcer Management — Sacral wound care per orders (Mepilex Border Sacrum q3 days + PRN). Comprehensive skin assessment each visit — every bony prominence. Braden reassessment weekly. Repositioning schedule enforcement and documentation. Nutrition optimization for wound healing (protein, Vitamin C, zinc). Moisture management (incontinence care, barrier cream). Consult wound care MD for any deterioration.

Neurogenic Bladder/Catheter Management — Foley catheter care per protocol. Monitor for CAUTI signs (new fever, cloudy/malodorous urine, increased spasticity, worsening AD). Catheter change per schedule (every 4 weeks or PRN). I&O monitoring. Urine culture only if symptomatic (not routine for chronic catheter). Catheter obstruction/kinking is the #1 trigger for AD in this patient — ensure free drainage AT ALL TIMES.

Neurogenic Bowel Program — Bowel program MWF: digital stimulation + bisacodyl suppository, seated on commode chair if tolerated, consistent timing. Monitor for fecal impaction (AD trigger). Document bowel patterns. Adjust regimen per results. Fecal impaction is the #2 trigger for AD in this patient.

Autonomic Dysreflexia Surveillance — AD protocol posted at bedside, in wheelchair bag, and in bathroom. All caregivers (mother, DeShawn, HHA) must be trained and demonstrate competence in AD recognition and intervention. BP monitoring at each visit and PRN for symptoms. AD is life-threatening — this cannot be overstated.

Anticoagulation Management — Warfarin monitoring (INR 2x/week; target 2.0-3.0). Multiple drug interactions with PEG medications — warfarin dosing may fluctuate. Assess for bleeding. Coordinate with PCP/hematology.

Pain Management — Neuropathic pain assessment using appropriate scale (DVPRS or NRS). Opioid use tracking (oxycodone PRN). Non-pharmacologic interventions (positioning, distraction, music therapy, relaxation). Coordinate with Dr. Patel. Baclofen pump evaluation follow-up.

Psychiatric Safety — C-SSRS at every visit. PHQ-9 biweekly. Monitor behavioral indicators (withdrawal, refusing care, talking about death, giving away possessions, "good-bye" statements). Document and report any concerns immediately. Safety plan review with patient and caregivers. Coordinate with Dr. Kovacs.

Caregiver Training & Support — Mother Angela is the backbone of this care plan. She is experiencing severe caregiver burnout. If Angela collapses, this patient's entire home care plan fails. Priorities: (a) train Angela AND DeShawn on ALL critical skills (vent troubleshooting, manual bagging, suctioning, AD management, trach emergency, PEG management, bowel program, catheter care, medication admin); (b) assess Angela's physical and mental health at each visit; (c) connect Angela with caregiver support group and respite services; (d) explore In-Home Supportive Services (IHSS) for additional paid caregiving hours; (e) explore Regional Center/SCVMC SCI outpatient resources.

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

Goal Target

- 1 Zero ventilator-related adverse events 08/12/2025
- 2 Successful vent weaning trials to ≥ 1 hour off-vent during daytime 08/12/2025
- 3 Trach downsized from #8 to #6 (per pulmonology) 08/12/2025
- 4 Sacral pressure ulcer fully epithelialized (closed) 08/12/2025
- 5 No new pressure ulcers 08/12/2025
- 6 Weight stable or gaining (target ≥ 155 lbs) 08/12/2025
- 7 Albumin ≥ 3.0 (from 2.6) 08/12/2025
- 8 Zero AD episodes requiring pharmacologic intervention 08/12/2025
- 9 Zero UTIs 08/12/2025
- 10 Bowel program effective (BM on program days with no incontinence between programs) for $\geq 80\%$ of cycles 08/12/2025
- 11 INR stable within range for $\geq 80\%$ of draws 08/12/2025
- 12 PHQ-9 decreased to ≤ 17 (from 22) 08/12/2025
- 13 Zero suicidal ideation episodes requiring crisis intervention 08/12/2025
- 14 Patient operates wheelchair with sip-and-puff independently in home 08/12/2025
- 15 Patient uses eye-gaze communication device for functional communication x 30 min sessions 08/12/2025
- 16 Caregiver (mother + DeShawn) demonstrate independent competence in: vent emergency, suctioning, trach care, AD protocol, PEG management, bowel program, catheter care 07/15/2025
- 17 IHSS application submitted and hours authorized 07/31/2025
- 18 Angela connected with caregiver support services 07/15/2025
- 19 Zero falls, zero ED visits, zero readmissions 08/12/2025
- 20 Patient verbalizes 3 personally meaningful goals for the next 6 months (adjustment to disability; building motivation) 08/12/2025

PATIENT SIGNATURES — EVALUATION 6

Patient Signature: _____ Date: _____

(Patient signed using adaptive grip pen holder on left hand with assistance — verified understanding verbally; patient alert, oriented, competent)

Print Name: Darius Malik Thompson

Mother / Health Care Agent Signature: _____ Date: _____

Print Name: Angela Thompson

Clinician Signature: _____ Date: _____

Patient / Representative Signature

Date

RN Clinician Signature

Date / Time